

PATIENT MEDICAL PROFILE: RAKESH PATEL

General Information:

- Full Name: Rakesh Patel
- Age: 62 years old
- Gender: Male
- Nationality: Indian
- Primary Language: Gujarati, Hindi, English
- Contact Number: +91 99887 76655
- Emergency Contact: Amit Patel (Son) - +91 99887 76654

Clinical Presentation:

- Chief Complaint: High-grade fever with shaking chills (rigors), pain/burning during urination (dysuria), increased urinary frequency, and progressive confusion/lethargy.
- Duration: Symptoms started 2 days ago, but mental confusion and lethargy started in the last 24 hours.
- Severity: Patient is poorly oriented to time and place, responding slowly to verbal commands.

Medical History:

- Chronic Conditions: Benign Prostatic Hyperplasia (BPH, diagnosed 3 years ago), Chronic Kidney Disease (CKD Stage III, baseline creatinine 1.6 mg/dL).
- Surgical History: None.

Current Medications:

- Tamsulosin 0.4 mg orally once daily at bedtime (for BPH)

Allergies:

- Sulfa drugs (Bactrim/Co-trimoxazole causes severe hives and swelling)

Triage Vital Signs:

- Blood Pressure: 94/58 mmHg (Hypotension, MAP 70 mmHg)
- Heart Rate: 115 bpm (Tachycardia)
- Respiratory Rate: 24 breaths per minute (Tachypnea)
- Temperature: 103.1°F (39.5°C) (Hyperthermia)
- Oxygen Saturation (SpO2): 93% on room air

Triage Assessment & Emergency Department Protocol:

- Suspected Diagnosis: Sepsis secondary to Urinary Tract Infection (Urosepsis).
- Recommended ESI Triage Level: ESI Level 2 (Patient meets SIRS/qSOFA criteria: RR $\geq$ 22, altered mental status, and borderline hypotension; high risk for septic shock).
- Required Immediate Interventions (Sepsis Bundle):
 1. Establish two large-bore IV lines immediately.
 2. Initiate rapid IV fluid resuscitation: 30 mL/kg Normal Saline or Lactated Ringer's (approx 2 Liters for his weight).
 3. Draw blood cultures (2 sets from different sites) and collect a clean-catch or catheterized urine sample for urinalysis and urine culture BEFORE administering antibiotics.
 4. Measure venous lactate level.

5. Administer broad-spectrum IV antibiotics (e.g., Ceftriaxone 2g IV or Meropenem 1g IV due to CKD, avoiding nephrotoxic drugs) as soon as cultures are drawn.

- Handoff Instructions: Closely monitor blood pressure every 15 minutes. Watch for signs of worsening tissue perfusion (delayed capillary refill, cold extremities). Keep physician updated on fluid balance (intake/output).